

I: Yeah, okay, so then we can start with the actual questions. The first question is about familiarity with AI. Could you describe how familiar you are with AI technologies in general?

P: Not really familiar. It's like 3 months since I discovered ChatGPT and I started to use it. I'm very new to this.

I: Could you also give a score on a scale of one to ten of how familiar you think you are?

P: Two.

I: Okay, and has your organization introduced any AI tools or systems in your day to day?

P: No, not yet, but we are discussing it.

I: But there has been something you're thinking of.

P: Yeah, it has been something we have discussed in meetings, but it's only a point of discussion. It's not really something we use.

I: Okay, well then we can go to the next question. This is for the diagnostics and assessment use. What are your thoughts on AI being used in diagnostic or assessment processes?

P: What do you mean with diagnostics?

I: For example, you have intake meetings, right? You could use AI possibly to help in those kinds of meetings.

P: Yeah, in intake meetings. And the question was?

I: What are your thoughts on AI being used in diagnostics or assessment processes?

P: I think that would be very nice, especially in intake sessions, because we talk about a lot, right? And we don't really have time to process all this information and to make a nice report on it, because that's not something we get money for via the health care, the insurances. That would be great. And I know a couple of general practitioners that use it, and it also helps us a lot when they use it because they don't have a lot of time as well. But the AI makes a really good summary of their sessions. I would be all for it.

I: So the main purpose you think it could have is summarizing the interview process, right?

P: Yeah, summarizing, that would be great.

I: And have you considered using it for maybe preliminary screening? Even before coming to talk with a psychologist, you could let them communicate with an AI to screen what the problems are, right?

P: Yeah, that would be good as well, because we see with screening that a lot of people that don't really belong in our section ,they belong in the more specialized one ,get through. I don't know how, to be honest, because we do questionnaires, but we do that when they are already in. So if you have an idea, that would be great.

I: Summarizing patient information ,I think we can move on to the next one. This is mainly for monitoring and progress tracking, so how do you view the role of AI in tracking patient progress over time?

P: How would I imagine that?

I: What is your opinion on the possibilities and possible risks?

I: What do you think you are doing now for progress tracking? Or is it something that's not useful?

P: It could be useful, but there's always a risk, right? That's what we are discussing as well with privacy, because we attach so much value to that. But I think for progress tracking, it would be great. We are doing questionnaires now: at the beginning, in the middle, and at the end we let clients fill out three questionnaires. But that's it. I'm not sure what the possibilities are with AI progress tracking.

I: I mean, they're obviously working on a lot of different tools, right? I don't want to say too much, because I don't want to influence your responses, but the main goal of this study is to understand the possible use cases for AI and of course if there are some gaps between what people working on AI think and what psychologists think. But there are always limitations in making these things, right?

P: Yeah.

I: What concerns, if any, would you have about AI tracking? That's mostly privacy, right?

P: Yeah, I think that's the biggest part, and also that clients don't agree to it. So you have a bit skewed data when some clients agree and others don't.

I: How are you going to do this?

P: You can do it the old-fashioned way, but...

I: Do you think clients would not be open to talking with an AI for monitoring purposes, or other purposes at all?

P: I think it depends, right? I think younger clients may be open to it, but older clients are a bit skeptical, so they may not be open to it.

I: Have you talked with any patients about this, or is this just your own idea?

P: No, this is just my own idea. I have not talked with any patients about this.

I: Okay, then we can move on to the next question. This is for administrative and practical support. How do you feel about AI generating drafts or reports or assisting with scheduling and reminders?

P: Also reminders for clients, for instance?

I: Yeah, reminders for psychologists or reminders for clients.

P: Sounds good. Because we are discussing this, right? Because a lot of clients, at least in this period of the year, forget their appointment or they just call in sick.

I: Is that specific for this time of the year?

P: Yeah, we had something like Blue Monday last week, so at this point of the year they have new year's resolutions and then after a month they slip up and motivation goes down the drain. That's kind of typical for this part of the year. But when they call in sick, it's a gray area, so we lose money. When you have more ways to keep a client involved throughout the week, or give them reminders that their appointment is coming up, I think it would be very good for motivation.

I: Your planning and scheduling, is that done by someone else, or are you handling the planning of meetings with clients yourself?

P: Yeah, we have an office manager.

P: She is kind of scheduling the meetings, but I'm doing it as well with clients in the room. I don't think AI would be any added value there, but I can be wrong. I think that goes pretty okay, and there's always an important part of treatments ending early or prolonging the treatment, so you can get gaps in the schedule. It's complex to always have meetings right after each other with no gaps, because it's changing every day with people being sick, treatments ending early, or people just not being available for intake conversations. I think there could be some added value there, but I'm not sure how.

I: Because the actual appointments are very dynamic, right? You have people making an appointment a week before and then canceling two days before.

P: Yeah, could be. We have a standard of 12 sessions, so it's pretty short. But it's always possible that someone cancels on the day itself, and when someone is sick you can't really say you're going to send them a bill. It's kind of a gray area. We always tell patients to call 48 hours before, but that's not always possible when you can't really see in the future if you're going to be sick.

I: And the 12 sessions, is that for all cases?

P: Yeah, because in the intake conversation we introduce that we have what's called in Dutch "Kortdurend behandelen," **short term treatment**. I'm not sure how you say this in English. That's kind of our standard and it can be very effective because it also helps with motivation. But I do a lot of variation. In the last four sessions I take one week in between, for example, when a client is doing better, so they can practice more. But then you have gaps, right?

I: Every other week, right?

P: Yeah, every other week. I don't know if that's an answer to your question.

I: Any answer is the correct answer. So you normally do 12 sessions, and I'm guessing in the beginning they need more help, so the meetings are closer together, and then at the end they start having some gaps. That leaves some opportunity to schedule better.

P: Yeah, true. And I think that's where AI can come in handy, to do something with this, right?

I: Because it's probably a lot of managing on your part and on the office manager's part. There might be something helpful there.

P: Yeah.

I: Do you have any concerns about accuracy and liability in that regard?

P: Yeah, of course. At least in the beginning, I'm not sure how accurate it's going to be, but you have this feeling of not really being in control, that there's some program doing this for you and you have to pick out the right things. I think that would be a bump you have to get over, to just see it as an experiment and see how it goes.

I: We can move on to the next part. This is for psychoeducation and between-session tools. What are your thoughts on using AI for psychoeducation or therapeutic exercises between sessions? Are you doing anything in between sessions already, like homework or other tools clients might use between therapy sessions?

P: Yeah, I always do this. It's very important because the biggest change happens outside of the sessions. I can say a hundred different things, but when you don't do anything between sessions then it's not going to work. Maybe that's a bit strict, but there's definitely some truth in that. It could help, as long as it doesn't contradict what I'm saying, because then you have a problem. You have to know what the AI is going to say, because otherwise we may be saying similar things... but I think it could be good. You have protocols, right, you know this.

I: There are different types of AI and also very different types of applications you could use it for. But this is a possible limitation.

P: I think it can be confusing, and also a lot of time investment. I see a lot of clients with burnout or depression, and you want to be a bit wary of the amount of work you give these clients. When they also have to talk with an AI bot, it could be a lot. A lot of clients think a lot, mostly negative

thoughts, that's why they come to a psychologist. So when they start to ruminate with an AI bot, that could be troubling. You can spend four hours talking with an AI bot, asking all the questions you have. It has to be carefully made for those types of clients.

I: What are the main kinds of homework you give your clients?

P: The homework I mostly give is challenging negative thoughts and putting a more helpful or more positive thought alongside the negative thought, to kind of arrive at a more nuanced thought in the end. There are a lot of forms for that, so I give those most.

I: So that's mostly for people with negative thoughts, where they recognize the negative thought and then try to reframe it or come up with some other explanation or feeling that could replace or counter the general negative idea?

P: Yeah, it's not really replacing, but it's okay to have the negative thought. You can nuance it.

I: So it's not only about the negative thought itself.

P: True, that's cognitive behavioral therapy. I do that a lot, and I also do some things with acceptance and commitment therapy. There's also homework around looking into your values and having aimed action towards your values, stuff like this.

I: Do you think, given what you were saying about the possibility of AI contradicting your views, what would be the right course of action in that case? Because for you to know that the AI is contradicting you, either your client has to tell you or you might be forced to monitor what the AI is telling the client, which would create more work and possibly some privacy problems. There are definitely some risks and limitations there.

P: Yeah, the biggest issue is time in psychology. The work pressure is pretty high because of insurances, right? We can't send a bill for everything to the insurance company. So I can't really monitor everything.

I: That is true. Because you're saying that maybe monitoring would be needed, but it could also be a problem that monitoring does not fall under insurance, right?

P: No, it doesn't.

I: So either insurance has to change, which is probably very hard. Insurance is a very big problem.

P: That's very optimistic.

I: Then there has to be a lot of changes in mental health care. But it is possible, of course. Monitoring AI in its early stages most likely will be needed, and it's very hard to get coverage with insurance for that.

P: In basic healthcare it is hard, but in specialized care I think there's more room for it because there's also more monitoring in general, and you can explain it as diagnostics. For example, by sending clients some kind of overview. You need direct client contact in order to get it insured, but in specialized care there's more room because you have a bigger diagnostic package. So it's not impossible, you just have to think of a smart way to explain it to the insurance companies. And also in basic healthcare, but you have to think of a clever way.

I: Clever ways of reframing the monitoring.

P: Yeah.

I: Now we'll go into risks and limitations. What risks or concerns do you have about using AI in clinical work? We've already mentioned some, but if there's anything else or anything you want to elaborate on.

P: I think we spoke about a lot, right? I think the biggest thing is that you have to be very careful about mentioning names and things like that, because there's always a possibility of information being leaked. If that happens once, you're screwed as a practice. That's a big reason why many practices don't really use it, because a lot of psychologists have this feeling that it can go wrong once and then they are done.

I: So during this conversation, the main risks you mentioned, correct me if I'm wrong, are privacy, the conflict between possible ideas the AI might have and what you've agreed on with your client, and then there was one more I think.

P: Also whether the summarizing is correct, for the intake sessions. And also the monitoring, right?

I: This is also something we talked about. So the client perspective and therapeutic alliance. How do you think clients respond to AI being used in their treatment? We already mentioned that some people might not be willing to participate while others are. Do you have any more thoughts on that?

P: Yeah, it differs for each client. From my own perspective, if a psychologist asked me if they could use AI, I think I would be fine with it, though I might be a bit hesitant about what I say in conversations, maybe holding back information, partly because I'm pretty new to AI. But I think there are people who know way more about AI who would think it's absolutely fine. Also, we always ask if we can contact the general practitioner, and 99 percent of people say yes, so they're not really bothered by us possibly sharing some information with them as long as it can help. I think people are also pretty open when you explain it in the right way.

I: I think that's very true. Some people might be hesitant, but if you have a clear way of explaining what you're actually using it for it makes a difference. If you say "can we use your data" or "can we record the audio and feed it into an AI" without really explaining what's going on, people might be more hesitant. But if you say "we will record the audio, delete it afterwards, and just use it to make a summary so it's easier to keep track of your progress," that explanation

is probably more acceptable. So there's definitely a factor where you need to explain why and how you're doing things.

P: Yeah, because it's not really a normal thing at this point. It's still new, so you have to explain it.

I: Could you also give a score of 1 to 10, where 1 that clients don't respond well and 10 being they respond well

P: Maybe a 6, but it depends on the group and how you explain it.

I: Last question. This is about professional identity, autonomy and future conditions. Which aspects of your work should remain strictly human, and what would you need to feel confident about using AI safely in practice?

P: I think client-psychologist contact has to stay human, though not fully human in the sense that you can do some things online as well, which can add to your treatment. But making summaries of intake sessions and things like that would be very nice to offload, because I have to find time in between sessions to do this and it's not something that is insured or that we can make money with. So that would be a big win.

I: So the main conversation should still stay human, right?

P: Yeah.

I: Have you had any online meetings with your clients as well?

P: Yeah, I have.

I: Do you see a clear difference between in-person sessions and online sessions?

P: Yeah, I see a pretty big difference, because just like now I don't see a lot of non-verbal communication. That's just my preference, because I know practices that do full treatment online, and science is also backing that for less severe conditions the results are equal for online and in person. But my preference is to do it in person, because online I don't see everything.

I: So you think online there is some chance for detachment, like you just don't feel the emotions in the room?

P: Yeah, that's true. And also because you're not really looking at each other, so it's a bit harder to get the connection you would get in person. But that's just preference, and there might be people who prefer online and people who prefer in person. I know some practices in the Netherlands that combine online and in-person treatment. They have an online platform where they do exercises with clients, and those clients also have sessions in person with more senior psychologists. They also do full treatments online if the case is not very severe.

I: So you also think less severe cases would maybe be easier to do online, while more severe cases might benefit more from in person?

P: Yeah, because there are also benefits to online: you're in your own safe space, you don't have to travel. And from what I know in the research, more severe cases benefit more from in-person than online, but for mild or light depression the outcome is equal. It's interesting.

I: For sure.

P: You can look it up. It's interesting.

I: Is there anything you want to add?

P: No, I think we talked about a lot.